

***Technical
Writing***



Technical Writing

Lecture (8)

By

Dr. Ahmed Salah Abou Taleb

Faculty of Engineering - Fayoum University

1st Semester

2025 - 2026

What is SBAR?

2

- SBAR technique **provides** a framework for communication **between** members of the health care team about a patient's condition.
- SBAR is a structured method for **communicating** critical information that requires **immediate** attention and action.
- SBAR **improve** communication, **effective** escalation and **increased** safety.
- SBAR has 4 steps:
 - ▣ Situation
 - ▣ Background
 - ▣ Assessment
 - ▣ Recommendation..

What is SBAR?

3

S	Situation
B	Background
A	Assessment
R	Recommendation

Why Use SBAR?

4

- To **reduce** the barrier to **effective** communication **across** different disciplines and levels of staff.
- SBAR **creates** a shared mental model **around** all patient handoffs and situations **requiring** escalation, or **critical** exchange of information (handovers)
- SBAR is memory **prompt**; **easy** to remember and **encourages** prior preparation for communication.
- SBAR **reduces** the incidence of **missed** communications..

How Can SBAR help me?

5

- **Easy** to remember.
- **Clarifies** what information needs communicating quickly.
- **Points** to action.

Prevents “hinting and hoping”..

Uses & Settings for SBAR

6

- **Inpatient** or **outpatient**.
- **Urgent** or **non urgent** communications.
- **Conversations** with a physician.
 - Particularly **useful** in nurse to doctor communications.
 - Also **helpful** in doctor-to-doctor consultation.
- **Discussions** with allied health professionals.
 - e.g. Respiratory therapy.
 - e.g. Physiotherapy.
- **Conversations** with peers.
 - e.g. Change of shift report.
- **Escalating** a concern .
- **Handover** from an ambulance crew to hospital staff ..

SBAR

7

Situation

- **Identify** yourself the site/unit you are calling from.
- **Identify** the patient by name and the reason for your report.
- **Describe** your concern.
- **Describe** the specific situation about which you are calling, including the patient's name, consultant, patient location, resuscitation status, and vital signs..

SBAR

8

S

Situation

Identify yourself & location

Identify patient (name, age, sex)

State diagnosis (suspected or definitive)

State reason for transfer or handover

(e.g. unavailable diagnostics or therapeutics)

☐
☐
☐
☐

"This is Lou, a registered nurse on Nightingale Ward. The reason I'm calling is that Mrs Taylor in room 225 has become suddenly short of breath, her oxygen saturation has dropped to 88 per cent on room air, her respiration rate is 24 per minute, her heart rate is 110 and her blood pressure is 85/50."

SBAR

9

Background

- **Give** the patient's reason for admission.
- **Explain** significant medical history.
- **Overview** of the patient's background: admitting diagnosis, date of admission, prior procedures, current medications, allergies, pertinent laboratory results and other relevant diagnostic results.
- For this, you **need** to have collected information from the patient's chart, flow sheets and progress notes..

SBAR

10

B

Background

Admission date ☐Relevant past medical & surgical history ☐Recent changes in status (ABCDE findings/interventions) ☐Relevant labs & imaging ☐Recent vital signs ☐Management or interventions provided ☐

(e.g. O2, infusions, antibiotics, procedures)

Relevant psychosocial factors ☐

"Mrs. Taylor is a 69-year-old woman who was admitted from home three days ago with a community acquired chest infection. She has been on intravenous antibiotics and appeared, until now, to be doing well. She is normally fit and well and independent."

SBAR

11

Assessment

- Vital signs.
- Clinical impressions, concerns.
- You **need** to think critically when informing the doctor of your assessment of the situation.
- This means that you have **considered** what might be the underlying reason for your patient's condition..

SBAR

12

A

Assessment

State the diagnoses or conditions (if diagnostic uncertainty) ☐**State severity of illness** (stable or critical) ☐**State patient trajectory** (worsening or improving) ☐**Report response to interventions provided** ☐

"Mrs. Taylor's vital signs have been stable from admission but deteriorated suddenly. She is also complaining of chest pain and there appears to be blood in her sputum. She has not been receiving any venous thromboembolism prophylaxis."

"I'm not sure what the problem is, but I am worried."

SBAR

13

R Recommendation

- **Explain** what you need - be specific about request and time.
- **Make** suggestions.
- **Clarify** expectations.
- **Finally,**
 - ✓ what is your recommendation?
 - ✓ what would you like to happen by the end of the conversation with the physician?
 - ✓ Any order that is given on the phone needs to be repeated back to ensure accuracy..

SBAR

14

R

Recommendation

State your recommendations & concerns

(e.g. transfer for specialist consult or frequent monitoring)

☐

State timeline for recommendations

(e.g. transfer or intervention needed in next 1 hour)

☐

State contingency plans

(e.g. If patient transfer is delayed, then I will...)

☐

"Would you like me get a stat CXR? and ABGs? Start an IV? I would like you to come immediately"

SBAR

15

S

Situation – patient's /client's details, identify reason for this communication, describe your concern

B

Background – relating to the patient/ client, significant history, this may include medications, investigations/ treatments

A

Assessment – what is your assessment of the patient/ client or situation, this can include clinical impression/ concerns, vital signs/ early warning score

R

Recommendations – be specific, explain what you need, make suggestions, clarify expectations, confirm actions to be taken

SBAR Case Study # 01

16

"This is Lou, a registered nurse on Nightingale Ward. The reason I'm calling is that Mrs Taylor in room 225 has become suddenly short of breath, her oxygen saturation has dropped to 88 per cent on room air, her respiration rate is 24 per minute, her heart rate is 110 and her blood pressure is 85/50."

"Mrs. Taylor is a 69-year-old woman who was admitted from home three days ago with a community acquired chest infection. She has been on intravenous antibiotics and appeared, until now, to be doing well. She is normally fit and well and independent."

"Mrs. Taylor's vital signs have been stable from admission but deteriorated suddenly. She is also complaining of chest pain and there appears to be blood in her sputum. She has not been receiving any venous thromboembolism prophylaxis."

"Would you like me get a stat CXR? and ABGs? Start an IV? I would like you to come immediately"..

SBAR Case Study # 01

17

S	✓ Lou, RN, on Nightingale Ward. ✓ Mrs. Taylor in room 225, a 69-year-old woman ✓ Community acquired chest infection. ✓ She is normally fit and well and independent.
B	✓ She was admitted from home three days ago ✓ Short of breath, SpO₂88%, RR24/min, HR110bpm, BP85/50mmHg. ✓ She has been on intravenous antibiotics and appeared, until now, to be doing well.
A	✓ deteriorated suddenly in vital sign. ✓ Chest pain complained. ✓ Blood in her sputum. ✓ Not receiving any venous thromboembolism prophylaxis.
R	I would like to: - CXR. - ABGs. - Start IV. Right away..

SBAR Case Study

18

Nurse to Nurse End-of-Shift Report

S	Mr. James is a 72-year-old male admitted yesterday with pneumonia. He's currently on 2L nasal cannula and remains hemodynamically stable.
B	He has a history of COPD, hypertension, and type 2 diabetes. He was started on IV antibiotics and steroids upon admission. High finger stick checks are every 4 hours. He's been on oxygen since arrival, and his SpO2 has ranged from 92–95%.
A	Lungs have scattered crackles bilaterally, and he has a productive cough. No fever, and his vital signs have been stable. He had some shortness of breath earlier but it improved with breathing treatments. His glucose has been elevated. I've given him three units most recently per his sliding scale.
R	Continue monitoring his respiratory status, encourage incentive spirometry, and administer scheduled albuterol nebulizers. His next antibiotic dose and finger stick are due at 2200. Call the provider if his SpO2 drops below 90% or if he shows signs of worsening respiratory distress.

SBAR Case Study

19

Nursing Communication With Family

S

Candice is doing better today. She has been tolerating fluids and is showing signs of improvement

B

She had been vomiting frequently at home and wasn't able to keep liquids down, so we admitted her and started IV fluids overnight

A

She is more alert and has had no vomiting since early this morning. She's started drinking small sips of water and apple juice without any issues. Her urine output has also improved.

R

We will keep monitoring her hydration and encouraging more oral fluids. If Candice keeps fluids down and maintains good energy levels, the doctor may consider discharging her later today or tomorrow. Let me know if you have any concerns, and I can update the provider for you.

SBAR Case Study

20

Nursing Assessment Changes Communicated to a Provider

S

Mrs. Carter is a 58-year-old female with a history of CHF admitted for pneumonia. She is now experiencing increased shortness of breath and worsening bilateral leg swelling.

B

She was admitted three days ago and treated with IV antibiotics and diuretics. She has a history of hypertension and CHF with a reduced ejection fraction. Her last BNP was elevated at 900.

A

Her respiratory rate is 24, SpO₂ is 89% on room air, and she has +3 pitting edema to bilateral lower extremities. Lungs have new crackles at the bases, and she reports feeling more fatigued.

R

Can we get a STAT chest X-ray to assess for fluid overload? And would you like me to put her on a nasal cannula for oxygen delivery in the meantime?

SBAR Case Study

21

Nursing Interview Questions

S

I had a post-op patient who suddenly became hypotensive after a routine knee replacement.

B

He was a 65-year-old male with a history of hypertension and diabetes. He had been stable but started feeling dizzy, and his blood pressure dropped to 85/50

A

I checked his vitals and found his heart rate was elevated, and he had cool, clammy skin. His surgical site was intact, but his urine output had decreased.

R

I called the provider and suggested a fluid bolus and additional monitoring. The provider ordered a rapid response, and we identified internal bleeding as the cause. My report helped us move quickly to care for the patient.

SBAR Case Study # 02

22

Chest Pain	
S	Mr. Jones is having increased dyspnea and complaining of chest pain on the left of chest.
B	He had her left hip replaced yesterday. He started complaining of chest pain about three hours ago. His pulse is 155, blood pressure 134 over 57, is restless and short of breath.
A	He may be having a pulmonary embolism, coronary artery disease, precordial chest pain, or a cardiac event.
R	I recommend you see him immediately and start him on O2.

SBAR Case Study # 03

23

Transfusion Reaction	
S	Mrs. Smith is a day case patient in hematology who is breathless and heart rate is at 130. She may be having a transfusion reaction.
B	Mrs. Smith came in at 6 AM this morning for a transfusion of 2 units of red blood cells. The first unit was given over 90 minutes and the second one started half an hour ago and is over halfway through. She developed dyspnea within the last 20 minutes, has an increased heart rate and elevated blood pressure.
A	Temperature is 37.1 degrees Celsius. Pulse is 130. Blood Pressure is 150/96. She's showing signs of circulatory overload. I have stopped the transfusion and started administering oxygen.
R	I recommend you come see the patient immediately.

SBAR Case Study # 04

24

Collecting Urine Specimens for Bacterial Culture

S

Mr. Williams's nurse is calling about the urine culture order.

B

Mr. Williams came in for an ordered urine culture for volvulus.

A

The patient is hypotensive and complaining of abdominal pain. He denies symptoms of a UTI, is afebrile and on heparin for his PICC-related DVT.

R

I recommend there's no need to collect urine for culture at this time or in the near future.

SBAR Case Study # 05

25

Elevated Creatinine	
S	Mrs. Johnson has an order form her doctor for a PE study.
B	Mrs. Johnson came in with difficulty breathing and right-side chest pain.
A	Mrs. Johnson's creatinine level is 3.1, which is far above the cutoff level for a PE study.
R	I recommend we change the PE study order to a VQ scan order.